Fax cover sheet

Seabrook Regional Hospital (fictional)

Case Management Department

FAX TRANSMITTAL

To: Illumination Health + Home - Recuperative Care Intake
Fax: 555-0199
From: Monique Barrett, MSW
Phone: 555-0120
Date: 09/23/2026 10:05
Pages (including cover): 10
Re: Recuperative care referral
Urgent [X] For Review [X] Please Reply [X]

Message

65M L hip fracture s/p repair, stable, ready for d/c.
Requesting recuperative care. H&P, op note, progress notes, labs attached.

CONFIDENTIALITY NOTICE

This facsimile contains confidential health information. If received in error, notify sender and destroy.

-- FICTIONAL DEMO RECORD - not a real patient --

ILLUMINATION Recuperative Care Referral Form

HEALTH + HOME

To refer a patient, please complete and submit this form with supporting documents via email to RECUP@ifhomeless.org or FAX to (888) 382-9551. For referral questions, please contact an Illumination Referral Coordinator at (888) 505-0855.

Referring Party

Referring Hospital: Seabrook Regional Hospital (fictional)
Dept/Floor: 4 West Ortho
SW/CM/RN: Monique Barrett, MSW
Phone/Pager #: 555-0120
Email: mbarrett@seabrookregional.example
Nursing Station #: 555-0121
Authorized by: Dr. A. Okafor
Phone #:

Patient Information

Patient Name: Walter Doe MRN: SR-2290415 Date of Birth (dd/mm/yyy): 11/02/1960
Gender: ☒ Male ☐ Female ☐ Transgender ☐ Other Ethnicity: White
Last 4 # of Social Security: xxx-xx- 0000 Insurance: Orange Coast Health Plan (fictional) Medi-Cal Preferred Language: English
Housing Situation: ☒ Currently Unhoused ☐ 4+ episodes of homelessness in the past 3 years

Hospital Admission Details

Chief Complaint Admitting Diagnosis:
L intertrochanteric hip fracture s/p intramedullary nail
Admit Date: 09/19/2026 Any Wounds? ☒ Yes ☐ No
If yes, explain the number, location, size and stage:
L hip surgical incision
Independent with Wound Care? ☒ Yes ☐ No If no, will Home Health be ordered? ☐ Yes ☒ No
Was PPD/TB Test or Chest X-Ray Performed? ☒ Yes ☐ No If yes, Date: 09/19/2026 Results: CXR no acute process

History of Health & Living Situation

Does the patient require or have... (Please circle Y or N and provide explanation accordingly.)

O2	Yes* ☒ No	Active Infection	Yes* ☒ No
*Explain		*Explain	
Self Administer All Meds	☒ Yes No*	Ambulatory	☒ Yes No
Continent of Bowel & Bladder	☒ Yes No	Assistive Device	☒ Yes* No
Colostomy/Ileostomy	Yes ☒ No	*Explain front-wheeled walker	
Foley Catheter	Yes ☒ No	Hospice	Yes* ☒ No
Diabetes	Yes ☒ No	*Explain	
Insulin	Yes ☒ No	Car	Yes ☒ No
Diabetes related Oral Meds	Yes ☒ No	Service Animal	Yes ☒ No

Behavioral and Mental Health Status

Does the patient have any limitations, behavioral or mental health challenges? ☒ No Yes

☐ Mental Health DX If checked, what type?
☐ Non-compliant ☐ Forgetful
☐ Cognitive Impairment ☐ Registered Sex Offender
☐ Other Explain none

Length of Stay Authorized in Recupertive Care: 14 Days

ILLUMINATION STAFF ONLY

Date Received: Reviewed by:
Approved: ☐ Yes* ☐ No** *Date: *Authorization #: **Reason for Denial:
Admission Date: Admission Time: Exit Date:
Extension Request: ☐ Yes ☐ No If YES, Additional Approval Date: by Total LOS

CalAIM Community Supports authorization request

CalAIM COMMUNITY SUPPORTS - AUTHORIZATION REQUEST

Service: Recuperative Care (Medical Respite)

Health Plan: Orange Coast Health Plan (fictional) - Medi-Cal

Member

Member Name: Walter Doe

DOB: 11/02/1960

Member ID: OCH-71938204

Referring Provider

Facility: Seabrook Regional Hospital (fictional)

Clinical

Primary Diagnosis: Intertrochanteric fracture of left femur, closed

ICD-10: S72.142A

Member is experiencing homelessness [X]

Member at risk of hospitalization/re-hospitalization [X]

Requested Start Date: 09/23/2026

Requested Days: 14

Plan Use Only

Status: Pending review

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Patient face sheet

Seabrook Regional Hospital (fictional) - PATIENT FACE SHEET

Patient: Walter Doe
MRN: SR-2290415
Account #: 99290415
DOB: 11/02/1960
Sex: Male
Preferred Language: English
Ethnicity: Not Hispanic or Latino
Marital Status: Single
Address: NO FIXED ADDRESS (homeless)
Mailing: 1234 Example Ave, Santa Ana, CA 92700 (shelter mail drop)
Phone: 555-0100 (message)

Encounter

Admit Date/Time: 09/19/2026 21:15 (ED arrival 09/19/2026 16:40)
Admit Type: Inpatient
Unit/Bed: 4W-418
Attending: Adaeze Okafor, MD

Insurance

Primary: Orange Coast Health Plan (fictional) - Medi-Cal
Subscriber: Self

Emergency Contact

None listed

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History and Physical

HISTORY AND PHYSICAL

Date of Service: 09/19/2026

Attending: Adaeze Okafor, MD (Orthopedic Surgery)

Chief Complaint

L hip pain after a fall, unable to bear weight

HPI

65 y/o M experiencing homelessness (sleeps in a tent near Example Creek x 3 years) tripped on a curb today and fell onto his L side. Unable to stand. No head strike, no LOC. L leg shortened and externally rotated. X-ray: closed L intertrochanteric femur fracture. Skin intact.

PMH

- Hypertension
- Osteoarthritis, knees

Allergies

NKDA

Social History

Homeless x 3 years. Former smoker. Denies alcohol and drug use.

Exam

T 98.6F HR 92 BP 146/88 RR 16 SpO2 97% RA. AOx4. L leg neurovascularly intact.

Assessment / Plan

Closed L intertrochanteric hip fracture. Plan intramedullary nail 09/20. Pain control. DVT ppx.

Signed: Adaeze Okafor, MD 09/19/2026 22:30

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Operative note

OPERATIVE NOTE

Date of Surgery: 09/20/2026
Surgeon: Adaeze Okafor, MD
Procedure: Cephalomedullary nail, L intertrochanteric femur fracture
Findings: Closed fracture, good reduction on fluoroscopy.
Closure: Layered closure. Skin closed with staples. Sterile dressing applied.
EBL: 150 mL. Complications: none.
Post-op: Weight bearing as tolerated L leg. PT/OT evaluate and treat.
Signed: Adaeze Okafor, MD 09/20/2026 12:45

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Orthopedic progress note

ORTHOPEDIC PROGRESS NOTE - POD#3

Date: 09/23/2026 07:30

S: Pain 4/10 with movement, controlled on oral meds. Eating well.

O: T 98.3F HR 82 BP 132/80 RR 16 SpO2 98% RA

L hip incision: staples intact, clean, dry, no erythema, no drainage.

Post-op Hgb stable.

A/P:

- s/p cephalomedullary nail L hip POD#3, uncomplicated. Weight bearing as tolerated.
- Dressing: dry gauze, change every 2 days or if soiled.
- Staple removal at 2 weeks post-op.
- DVT ppx: apixaban 2.5 mg PO BID x 35 days
- Medically stable for d/c

Signed: Adaeze Okafor, MD

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Invented for the Claude Impact Lab demo. Names, facilities and numbers are fictional.

Case management note

CASE MANAGEMENT NOTE

Date: 09/23/2026 09:20

Pt AOx4, pleasant and cooperative, no behavioral concerns per nursing.

Psychosocial screen: denies depression. Denies suicidal ideation. Denies homicidal ideation. No hallucinations. No psychiatric history. No history of conservatorship or FSP.

Substance use: denies alcohol and drug use.

Pt agreeable to recuperative care. Front-wheeled walker ordered for discharge.

Ortho follow-up: Seabrook Orthopedic Clinic (fictional) 10/04/2026 at 13:15 (confirmed) for wound check.

Outpatient PT referral: sent, not yet scheduled.

Signed: Monique Barrett, MSW

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Vitals, labs and microbiology

VITAL SIGNS - LAST 5 SETS

Date/Time	Temp (F)	HR	BP	RR	SpO2	O2
09/22 08:00	98.7	86	136/82	16	97%	RA
09/22 16:00	98.5	84	134/80	16	98%	RA
09/22 23:00	98.1	78	128/78	14	98%	RA
09/23 04:00	98.0	76	126/76	16	98%	RA
09/23 07:30	98.3	82	132/80	16	98%	RA

LABS

09/19	WBC 9.8	Hgb 13.6	Plt 251	Cr 1.0
09/22	WBC 8.4	Hgb 11.2 L	Plt 238	Cr 0.9

MICROBIOLOGY

09/20 05:30 MRSA nares PCR: NEGATIVE
09/23 06:10 SARS-CoV-2 rapid antigen: NEGATIVE

IMAGING

09/19 CXR: No acute cardiopulmonary process
09/20 L hip X-ray post-op: hardware in good position

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Discharge medication list

DISCHARGE MEDICATION LIST

Patient: Walter Doe MRN: SR-2290415

- Apixaban 2.5 mg PO twice daily x 35 days (NEW, DVT prevention)
- Oxycodone 5 mg PO every 6 hours PRN severe pain x 5 days
- Acetaminophen 1000 mg PO every 8 hours
- Amlodipine 5 mg PO daily

Wound Supplies

- Dry gauze, paper tape

DME

- Front-wheeled walker (ordered)

No antibiotics at discharge. No IV access at discharge.

Patient able to self-administer all medications: Yes

30-day supply dispensed from outpatient pharmacy: Yes

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